

Wed, Jul 1 at 12:03 AM

NDIS High Cost AT Report 계속 작성하는 중이야. 이것저것 많은 수정을 거쳐서 한번에 보기가 어려워 지금 이창에 수정사항 반영된 최종본 차례대로 불러와줄수 있어?
있으면 블록 1부터 시작해줘

현재 이 대화창에는 최종 통합본 전체 텍스트가 남아있지 않고, 이전 대화들에서 여러 차례 수정·재작성된 버전들이 섞여 있어서 제가 확정된 "최종본 블록 1"을 그대로 불러올 수는 없습니다.

특히 아래 수정사항들이 이후에 추가되었기 때문에 어떤 버전이 최종인지 확인이 필요합니다.

Independence 표현 수정

"M remains highly independent" 관련 수정

Activity level 낮음 반영

Falls risk 강조

0/5 (Physiotherapist) vs 3/5 (Neurologist) 근력 차이 반영

Section 17, 18 재작성

NDIS Reasonable & Necessary 연결 강화

High Cost AT 형식으로 전체 리라이팅

Claude 원본 구조 기반 재구성

그래서 지금 가능한 가장 정확한 방법은:

현재 작성 중인 최신 Word 파일을 업로드

또는

현재 사용 중인 블록 1을 붙여넣기

해주시면 제가:

지금까지 논의된 수정사항 전부 반영

0/5 vs 3/5 strength discrepancy 반영

NDIS reviewer 관점으로 강화

High Cost AT 제출용 수준으로 정리

해서 실제 제출용 최종본 Block 1을 만들어 드릴 수 있습니다.

현재 상태에서는 예전 여러 버전 중 어느 것이 사용 중인 최신본인지 확인할 수 없어서, 임의로 불러오면 오히려 최신 수정사항이 누락될 가능성이 있습니다.

최신 Word 파일이나 현재 Block 1 내용만 올려주시면 바로 최종본 Block 1부터 차례대로 다시 정리해드리겠습니다.

NDIS_L300Go_AT_Report_Formatted.docx

Document

좋아. 방금 업로드한 문서를 확인해보니 현재 네가 작업 중인 버전은 68페이지짜리 High Cost AT Report이고, 이 문서가 우리가 계속 수정해 온 베이스 문서야.

먼저 검토 결과부터 말하면 현재 Block 1(Executive Summary)은 아직 우리가 마지막에 합의했던 수정사항들이 모두 반영된 상태는 아니야.

특히 아래 부분들은 수정 필요.

수정 필요 1

현재 문서:

M has lived with Multiple Sclerosis for approximately five years and experiences progressive neurological impairment affecting her mobility and functional independence.

"functional independence" 표현이 들어가 있음.

우리가 마지막에 논의했던 방향은

independence 강조 ↓
activity restriction 강조 ↑
participation restriction 강조 ↑
compensatory effort 강조 ↑

였음.

수정 필요 2

현재 문서:

Although M remains able to mobilise using a four-wheeled walker...

이 표현은 괜찮지만

바로 뒤에

mobility is maintained through substantial compensatory effort

는 유지하면서

activity limitation을 더 강하게 연결하는 것이 좋음.

수정 필요 3

현재 문서:

Complete loss of voluntary right ankle dorsiflexion

이 부분은 Physio 평가값(0/5)만 반영됨.

우리가 나중에 논의한 내용:

Physio: DF 0/5

Neurologist: DF approximately 3/5

두 평가가 다른 시점에서 측정되었음을 설명해야 함.

현재 버전은 reviewer가

"그러면 왜 Neurologist는 3/5 인가?"

라고 질문할 가능성이 있음.

현재 최신본 기준 Block 1

(Executive Summary 전체를 제출용 최종 버전으로 수정)

1. Executive Summary

Purpose of Report

This report has been prepared to support funding approval for the purchase of a Bioness L300 Go Functional Electrical Stimulation (FES) System under the National Disability Insurance Scheme (NDIS) as High Cost Assistive Technology.

M is a 35-year-old woman with Multiple Sclerosis (MS) who experiences significant mobility limitations associated with right-sided foot drop, lower limb weakness, gait instability, fatigue, reduced balance confidence and restricted community participation. These impairments substantially affect her ability to safely and efficiently access her home and community environments and limit her participation in meaningful daily activities.

A comprehensive assessment process was completed, including:

- Detailed orthotic and functional assessment
- In-clinic Bioness L300 Go trial
- Eight-week community-based trial
- Three-week washout period following device return
- Physiotherapy outcome measurement
- Participant and carer feedback collection
- Neurologist review and endorsement

The purpose of this report is to determine whether the Bioness L300 Go represents the most appropriate, effective and reasonable assistive technology intervention to address M's disability-related mobility limitations and support achievement of her NDIS goals.

Participant Overview

M has lived with Multiple Sclerosis for approximately five years and experiences progressive neurological impairment affecting lower limb function, walking efficiency and community mobility.

Her condition has resulted in:

- Right foot drop during swing phase
- Reduced foot clearance
- Ankle instability
- Knee hyperextension during stance
- Walking-related fatigue
- Reduced standing tolerance
- Reduced walking endurance
- Reduced balance confidence
- Activity avoidance behaviours
- Reduced participation in community, family and social activities

Although M is able to mobilise using a four-wheeled walker, this mobility is achieved through substantial compensatory effort, careful activity pacing and avoidance of situations that increase fatigue, instability or falls risk.

Importantly, M routinely limits the frequency, duration and complexity of activities she undertakes in order to manage fatigue and reduce the likelihood of tripping, near-falls and mobility-related incidents. As a result, her functional limitations extend beyond gait impairment and significantly restrict participation across multiple life domains.

Assessment Findings

Clinical assessment identified the following neurological and functional impairments.

Neurological Impairments

- Right ankle dorsiflexion weakness (Physiotherapy assessment: 0/5)
- Right ankle dorsiflexion strength subsequently documented by treating neurologist as approximately 3/5 during review
- Right ankle eversion: 0/5
- Right ankle inversion: 1/5
- Right plantarflexion weakness

Moderate spasticity affecting plantarflexors

Mild spasticity affecting invertors

Functional Impairments

Foot drop during swing phase

Toe drag

Inversion during gait

Knee hyperextension during stance

Reduced walking speed

Reduced walking endurance

Reduced standing tolerance

Increased fatigue with activity

Increased risk of trips and near-falls

These impairments directly restrict M's ability to safely access her community, engage in daily activities and participate in family and social roles.

이 버전이 지금까지 논의했던

Independence 표현 수정

Low activity level 반영

Falls risk 강조

Activity avoidance 강조

0/5 vs 3/5 discrepancy 설명

NDIS reviewer 관점

을 반영한 실제 제출용 최종 Block 1이야.

다음 단계로는 Block 2 (Section 2 전체: Participant Profile and Disability Impact)를 현재 문서와 비교해서 수정사항 반영한 최종본으로 정리할 수 있어.

Mon, Jul 13 at 12:44 AM

Fatigue-Severity-Scale_copy.pdf

PDF

좋아. 방금 업로드한 FSS를 확인했어.

이 자료에 따르면:

평가일: 21/01/2026

FSS 1~9번 문항 모두 7점

총점: 63/63

평균점수: 7.0/7

VAFS는 약 6/10으로 표시됨

이건 사실상 최대 수준의 피로도를 의미해.

따라서 Appendix A의 FSS 부분은 기존 버전보다 훨씬 강하게 작성할 수 있어.

A10. Fatigue Severity Scale (FSS)

The Fatigue Severity Scale (FSS) was completed by the participant on 21 January 2026 and reviewed by the treating physiotherapist.

Baseline FSS Results

Measure Result

Total Score 63/63

Average Score 7.0/7

The participant scored the maximum rating of 7/7 on all nine FSS items, indicating severe fatigue affecting all measured domains of daily function.

Assessment findings demonstrated that fatigue:

- Reduced motivation and activity participation
- Was exacerbated by physical activity
- Interfered with physical functioning
- Caused ongoing difficulties in daily life
- Limited sustained physical activity
- Interfered with responsibilities and routine activities
- Was considered one of the participant's most disabling symptoms
- Negatively affected family, social and community participation

The findings indicate that fatigue represented a major disability-related barrier to mobility, endurance, participation and overall function.

Given the participant's diagnosis of Multiple Sclerosis, the severity of fatigue identified at baseline was considered clinically significant and consistent with the substantial activity restriction observed throughout assessment.

Although repeat FSS assessments were not completed during the community trial, participant reports, carer feedback and community trial observations indicated a reduction in fatigue-related activity restriction during active use of the Bioness system.

그리고 Appendix A의 날짜도 이제 수정 가능해.

A1 Assessment Overview

기존:

A comprehensive physiotherapy assessment was completed on [Insert Assessment Date]

수정:

A comprehensive physiotherapy assessment was completed on 21 January 2026 prior to commencement of the Bioness L300 Go community trial.

현재 Appendix A에서 남은 확인사항은 사실상 하나뿐이야.

Outdoor mobility를 어떻게 기록할지

초기 Physio 이메일에는

Independent with a 4-Wheeled Walker indoors

만 있고,

우리는 나중에

community mobility 매우 조심스럽게 수행

low activity level

활동 제한

이라는 정보를 추가했어.

그래서 A7 Mobility Status를 최종 제출용으로 다듬으려면:

Outdoor에서도 4WW 사용했는지, 아니면 indoor만 4WW였는지 알려주면 Appendix A를 완전히 최종본으로 확정할 수 있어.

좋은 포인트야.

실제 NDIS High Cost AT 보고서를 보면, Orthotist review notes 나 Community trial review notes 를 Appendix로 다 넣는 경우보다, 본문에서 충분히 설명하고 원본 증빙만 첨부하는 경우가 더 많아.

네 케이스는 이미:

- Block 5 (Clinical Assessment Findings)
- Block 8 (In-Clinic Trial Outcomes)
- Block 9 (Community Trial Methodology)
- Block 10 (Community Trial Outcomes)
- Block 14 (Washout Findings)

에 내용이 다 들어가 있음.

그래서 B와 C를 아래처럼 단순화할 수 있어.

APPENDIX A – PHYSIOTHERAPY ASSESSMENT AND OUTCOME MEASURES

Attachment A1

Physiotherapy Referral and Clinical Assessment Summary

Attachment A2

Physiotherapy Outcome Measure Summary Table

Attachment A3

Fatigue Severity Scale (FSS)

Attachment A4

Patient Specific Functional Scale (PSFS)

APPENDIX B – ORTHOTIC ASSESSMENT AND CLINICAL EVALUATION

Attachment B1

Orthotic Assessment Record

Attachment B2

Bioness L300 Go Configuration Record

(하나의 orthotic assessment form 또는 fitting form)

Purpose

To provide supporting clinical documentation relating to orthotic assessment, device selection and clinical evaluation of Functional Electrical Stimulation.

APPENDIX C – COMMUNITY TRIAL EVIDENCE

Attachment C1

Participant Community Trial Logbook Summary

(없으면 삭제 가능)

또는

Attachment C1

Community Trial Timeline

(1페이지)

Date Activity

Initial fitting Commencement of trial

8 Apr Review

21 Apr Review and optimisation

11 May Final review and device return

Washout 3 weeks

그런데 솔직히 말하면 이것도 없어도 됨.

내가 실제 제출한다면

Appendix B

Attachment B1

Orthotic Assessment Form

(내가 작성한 실제 orthotic assessment)

Appendix C

삭제

왜냐하면

Community Trial 증빙은 이미:

Appendix D

8-week Logbook

Washout Logbook

Appendix E

Participant Feedback

Appendix F

Mother Feedback

Appendix G

Neurologist Letter

Appendix H

Product Brochure

Quote

에 다 들어있기 때문이야.

내가 최종 제출본으로 추천하는 가장 깔끔한 구조

Appendix A

Physiotherapy Assessment and Outcome Measures

Referral/Assessment Summary

Outcome Measure Table

FSS

PSFS

Appendix B

Orthotic Assessment Record

Appendix C

Participant Logbooks

8 Week Trial Logbook

3 Week Washout Logbook

Appendix D

Participant Feedback

Appendix E

Carer Feedback

Appendix F

Neurologist Review Letter

Appendix G

Product Brochure and Quote

이 구조가 지금 네가 실제로 가지고 있는 자료만 사용하면서도, B와 C에 별도 "노트" 문서를 새로 만들지 않아도 되는 가장 깔끔한 구성이라고 생각해. 심사관 입장에서도 필요한 증빙을 찾기 쉽고, 중복도 최소화돼.

피지오 레퍼럴이나 어썬스먼트 썸머리도 대체할수 있는 방법 찾아줘. 레포트 자체에 포함된 내용들을 어펜덱스에 중복해서 포함시킬 필요 없을거 같아

나도 지금 다시 전체 구조를 보면 Appendix를 너무 많이 만들려고 했던 것 같아.

NDIS High Cost AT 심사관 입장에서 생각하면:

심사관은

본문 읽음

결과 확인

증빙자료 확인

순서로 봐.

이미 네 본문(Block 1~20)에

Physiotherapy findings

Strength

ROM

Spasticity

Gait findings

Outcome measures

Trial methodology

Trial outcomes

Washout findings

이 전부 들어가 있는데,

Appendix에서 같은 내용을 다시 설명하는 문서를 붙이면

"왜 같은 내용을 두 번 썼지?"

가 돼.

내가 실제 제출한다면

Appendix A도 삭제 가능

왜냐하면

이미 본문에 포함됨

Section 5 Clinical Assessment Findings

Section 8 In-Clinic Trial Outcomes

Section 10 Community Trial Outcomes

Section 14 Washout Findings

그래서 굳이

Physiotherapy Referral

Assessment Summary

같은 "내가 다시 작성한 문서"를 Appendix로 넣을 필요가 없어.

진짜 필요한 Appendix만 남기기

Appendix A – Outcome Measure Evidence

Attachment A1

Physiotherapy Outcome Measure Table

(피지오 이메일 원본)

Attachment A2

Fatigue Severity Scale (FSS)

Attachment A3

Patient Specific Functional Scale (PSFS)

왜 남기냐?

본문에 숫자는 들어가 있지만

심사관이

"원본 데이터 보여줘"

할 때 필요함.

Appendix B – Participant Trial Documentation

Attachment B1

8 Week Community Trial Logbook

Attachment B2

3 Week Washout Logbook

이것도 원본 증빙.

Appendix C – Participant Feedback

Attachment C1

Participant Feedback Statement

(네가 정리한 Quotes 1페이지)

Appendix D – Carer Feedback

Attachment D1

Mother / Primary Carer Feedback Statement

Appendix E – Medical Evidence

Attachment E1

Neurologist Letter

(지지서 포함이면 이것 하나면 충분)

Appendix F – Product Information and Quote

Attachment F1

Bioness L300 Go Product Brochure

Attachment F2

Supplier Quote

내가 추천하는 최종 구조
본문

Section 1 ~ 20

(모든 임상적 설명 포함)

Appendix A

Outcome Measure Evidence

Appendix B

Trial Logbooks

Appendix C

Participant Feedback

Appendix D

Carer Feedback

Appendix E

Neurologist Evidence

Appendix F

Product Brochure & Quote

이 구조의 장점은:

- ✔ 중복 거의 없음
- ✔ 심사관이 원본 증빙만 바로 확인 가능
- ✔ 페이지 수 10~15페이지 이상 감소
- ✔ "보고서"와 "증빙자료" 역할이 명확히 분리됨

실제로 네 케이스처럼 자료가 충분한 High Cost AT 보고서라면, 나는 Physiotherapy Referral Summary, Orthotic Assessment Summary, Community Trial Review Summary 같은 별도 Appendix 문서들을 전부 빼고, 원본 증빙자료만 Appendix에 넣는 방식으로 제출할 거야. 이게 가장 깔끔하고 전문적으로 보인다.

원본 증빙자료만 넣는 방식으로 어펜덱스 복사 붙여넣기 할수있게 작성해줘

아래 버전이 내가 실제로 NDIS High Cost AT Report 최종 제출본이라면 사용하는 Appendix 구조야.

핵심 원칙은:

본문(Sections 1–20) = 임상 분석, 해석, 근거

Appendices = 원본 증빙자료만 첨부

즉, 내가 작성한 Summary 문서나 Reasoning 문서를 Appendix에 다시 넣지 않고 실제 증빙자료만 첨부하는 방식이야.

21. APPENDICES

The following appendices contain supporting evidence referenced throughout this report.

The appendices have been limited to original supporting documentation, objective outcome measure data, participant records, medical evidence and supplier information. Clinical interpretation, analysis and recommendations are contained within the body of this report.

Appendix A – Outcome Measure Evidence

Attachment A1

Physiotherapy Outcome Measure Summary Table

(Including 10 Metre Walk Test, Timed Up and Go, Six Minute Walk Test, Dynamic Gait Index and Thirty Second Sit-to-Stand Test results completed by the treating physiotherapist)

Attachment A2

Fatigue Severity Scale (FSS)

Attachment A3

Patient Specific Functional Scale (PSFS)

Purpose

To provide objective outcome measure data collected throughout the assessment process.

Appendix B – Community Trial Documentation

Attachment B1

Eight-Week Community Trial Logbook

Attachment B2

Three-Week Washout Logbook

Purpose

To provide participant-recorded information regarding device use, daily activities, mobility experiences, fatigue, confidence and functional performance throughout the community trial and washout periods.

Appendix C – Participant Feedback

Attachment C1

Participant Feedback and Interview Responses

Purpose

To provide participant-reported experiences regarding the impact of the Bioness L300 Go System on mobility, participation, confidence and quality of life.

Appendix D – Carer Feedback

Attachment D1

Mother / Primary Carer Feedback Statement

Purpose

To provide independent observations regarding functional changes, participation and confidence observed during the community trial.

Appendix E – Medical Supporting Documentation

Attachment E1

Neurologist Clinical Review and Recommendation

Purpose

To provide supporting medical evidence regarding diagnosis, functional presentation and suitability for Functional Electrical Stimulation technology.

Appendix F – Product Information and Financial Documentation

Attachment F1

Bioness L300 Go Product Information

Attachment F2

Supplier Quotation

Purpose

To provide technical and financial information relating to the recommended Assistive Technology.

Appendix Summary

The attached appendices provide objective clinical evidence, participant-reported outcomes, carer observations, medical supporting documentation and supplier information which collectively support the recommendation for funding of the Bioness L300 Go Lower Leg and Thigh Components as a reasonable and necessary Assistive Technology support under the NDIS.

이 버전이면:

Physiotherapy Referral Summary ✕ 삭제

Orthotic Assessment Summary ✕ 삭제

In-Clinic Trial Summary ✕ 삭제

Community Trial Plan ✕ 삭제

Review Summary ✕ 삭제

Device Optimisation Summary ✕ 삭제

Washout Summary ✕ 삭제

전부 본문에만 남기고,

Appendix는 실제 원본 자료만 첨부하게 돼서 가장 깔끔하고 전문적인 최종 제출 패키지가 된다.